



FLUTINIDE Nasal Spray 50mcg/dose

DESCRIPTION: An opaque off-white slight viscous suspension containing Fluticasone Propionate 0.05% w/v.

ROUTE OF ADMINISTRATION: Nasal.

PHARMACODYNAMICS: Fluticasone propionate is a synthetic trifluorinated corticosteroid with anti-inflammatory activity. In vitro dose response studies on a cloned human glucocorticoid receptor system involving binding and gene expression afforded 50% responses at 1.25 and 0.17 nM concentrations, respectively.

PHARMACOKINETICS: Fluticasone propionate is poorly absorbed from the gastrointestinal tract and undergoes extensive first-pass metabolism; oral bioavailability is reported to be only about 1%.

INDICATIONS: For prophylaxis and treatment of allergic rhinitis.

RECOMMENDED DOSAGE:

Adults and children over 12 years of age: Two sprays into each nostril once a day, preferably in the morning. In some cases two sprays into each nostril twice daily may be required. Once symptoms are under control a maintenance dose of one spray per nostril once a day may be used. If symptoms recur the dosage may be increased accordingly. The minimum dose should be used at which effective control of symptoms is maintained. The maximum daily dose should not exceed four sprays into each nostril.

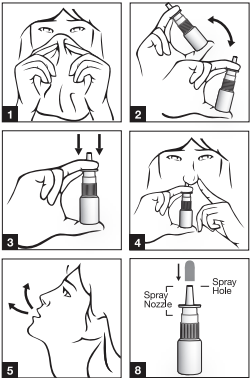
Elderly patients: The normal adult dosage is applicable.

Adolescents and children (4 years of age and older): A dose of one spray into each nostril once daily preferably in the morning is recommended. In some cases one spray into each nostril twice daily may be required. The maximum daily dose should not exceed two sprays into each nostril. The minimum dose should be used at which effective control of symptoms is maintained.

For full therapeutic benefit regular usage is essential. The absence of an immediate effect should be explained to the patient, as maximum relief may not be obtained until after 3 to 4 days of treatment.

HOW TO USE THE SPRAY

1. Blow the nose gently to clear nostrils.
2. Shake the bottle gently.
3. For first time use, hold the bottle upright and fill the pump by pressing the nozzle downwards and releasing it 5 times, until a fine spray is produced.
4. Close one nostril with finger. Hold the bottle upright and insert the nozzle into the nostril as far as is comfortable. Whilst breathing in gently through the nose with the mouth closed, press the bottle upwards once to deliver one spray.
5. Breathe out through the mouth.
6. Repeat steps 4 and 5 for the other nostril.
7. For patients aged 12 years and older, steps 4, 5 and 6 should be repeated once more to achieve the recommended dosage of 2 sprays into each nostril.
8. To keep the spray nozzle clean, wipe it carefully with a clean tissue after each use and replace the cap.



If the nasal spray has not been used for 2 weeks or more, it needs to be sprayed once into the air before use. The nozzle should be pointed away while doing this.

Always shake the bottle gently before use.

If the spray does not work and it may be blocked, clean it as follows. (See HOW TO CLEAN THE SPRAY). NEVER try to unblock it or enlarge the tiny spray hole with a pin or other sharp object because this will destroy the spray mechanism.

The nasal spray should be cleaned at least once a week or more often if it gets blocked.

HOW TO CLEAN THE SPRAY

1. Remove the cap and pull off the spray nozzle only.
2. Soak the dust-cap and spray nozzle in warm water for a few minutes, and then rinse under cold running tap water.
3. Shake or tap off the excess water and allow to air-dry.
4. Re-fit the spray nozzle.
5. Prime the unit as necessary until a fine mist is produced and use as normal.

CONTRAINDICATIONS: Hypersensitivity to any of its ingredients.

INTERACTIONS WITH OTHER MEDICAMENTS: Under normal circumstances, low plasma concentrations of fluticasone propionate are achieved after inhaled dosing, due to extensive first pass metabolism and high systemic clearance mediated by cytochrome P450 3A4 in the gut and liver. Hence, clinically significant drug interactions mediated by fluticasone propionate are unlikely. In an interaction study in healthy subjects with intranasal fluticasone propionate, ritonavir (a highly potent cytochrome P450 3A4 inhibitor) 100 mg b.i.d. increased the fluticasone propionate plasma concentrations several hundred fold, resulting in markedly reduced serum cortisol concentrations. Cases of Cushing's syndrome and adrenal suppression have been reported. The combination should be avoided unless the benefit outweighs the increased risk of systemic glucocorticoid side-effects.

In a small study using inhaled fluticasone propionate in healthy volunteers, the slightly less potent CYP3A inhibitor ketoconazole increased the exposure of fluticasone after a single inhalation by 150%. This resulted in a greater reduction of plasma cortisol as compared with fluticasone propionate alone. Co-treatment with other potent CYP3A inhibitors, such as itraconazole, is also expected to increase the systemic fluticasone propionate exposure and the risk of systemic side-effects. Caution is recommended and long-term treatment with drugs should if possible be avoided.

WARNINGS AND PRECAUTIONS: Local infections: Infections of the nasal airways should be appropriately treated but do not constitute a specific contra-indication to treatment with Flutinide Nasal Spray. The full benefit of Flutinide Nasal Spray may not be achieved until treatment has been administered for several days.

Care must be taken while transferring patients from systemic steroid treatment to Flutinide Nasal Spray if there is any reason to suppose that their adrenal function is impaired.

Although Flutinide Nasal Spray will control seasonal allergic rhinitis in most cases, an abnormally heavy challenge of summer allergens may in certain instances necessitate appropriate additional therapy.

Systemic effects of nasal corticosteroids may occur particularly at high doses prescribed for prolonged periods. These effects vary between patients and different corticosteroids.

Growth retardation has been reported in children receiving some nasal corticosteroid at licensed doses. It is recommended that the height of children receiving prolonged treatment with nasal corticosteroids is regularly monitored. If growth is slowed, therapy should be reviewed with the aim of reducing the dose of nasal corticosteroid, if possible to the lowest dose at which effective

control of symptoms is maintained. In addition, consideration should be given to referring the patient to a paediatric specialist.

Treatment with higher than recommended doses of nasal corticosteroids may result in clinically significant adrenal suppression. If there is evidence for higher than recommended doses being used then additional systemic corticosteroid cover should be considered during periods of stress or elective surgery.

Ritonavir can greatly increase the concentration of fluticasone propionate in plasma. Therefore, concomitant use should be avoided, unless the potential benefit to the patient outweighs the risk of systemic corticosteroid side effects. There is also an increased risk of systemic side effects when combining fluticasone propionate with other potent CYP3A inhibitors.

USAGE IN PREGNANCY AND LACTATION: There is inadequate evidence of safety in human pregnancy. Administration of corticosteroids to pregnant animals can cause abnormalities of foetal development, including cleft palate and intra-uterine growth retardation. There may therefore be a very small risk of such effects in the human foetus. It should be noted, however, that the foetal changes in animals occur after relatively high systemic exposure; direct intranasal application ensures minimal systemic exposure.

As with other drugs the use of Flutinide Nasal Spray during human pregnancy requires that the possible benefits of the drug be weighed against the possible hazards.

The secretion of fluticasone propionate in human breast milk has not been investigated. Subcutaneous administration of fluticasone propionate to lactating laboratory rats produced measurable plasma levels and evidence of fluticasone propionate in the milk. However, following intranasal administration to primates, no drug was detected in the plasma, and it is therefore unlikely that the drug would be detectable in milk. When Flutinide Nasal Spray is used in breast feeding mothers the therapeutic benefits must be weighed against the potential hazards to mother and baby.

ADVERSE EFFECTS: Adverse events are listed below by system organ class and frequency. Frequencies are defined as: very common ($\geq 1/10$), common $\geq 1/100$ and $< 1/10$, uncommon ($\geq 1/1000$ and $< 1/100$), rare ($\geq 1/10,000$ and $< 1/1000$) and very rare ($< 1/10,000$) including isolated reports. Very common, common and uncommon events were generally determined from clinical trial data. Rare and very rare events were generally determined from spontaneous data. In assigning adverse event frequencies, the background rates in placebo groups were not taken into account.

System Organ Class	Adverse Event	Frequency
Immune system disorders	Hypersensitivity reactions with the following manifestations:	
	Cutaneous hypersensitivity reactions	Very rare
	Angioedema (mainly facial and oropharyngeal oedema)	Very rare
	Respiratory symptoms (bronchospasm)	Very rare
	Anaphylactic reactions	Very rare
Nervous system disorders	Headache, unpleasant taste, unpleasant smell	Common
Eye disorders	Glaucoma, raised intraocular pressure, cataract These events have been identified from spontaneous reports following prolonged treatment.	Very rare
Respiratory, Thoracic & Mediastinal disorders	Epistaxis	Very common
	Nasal dryness, nasal irritation, throat dryness, throat irritation.	Common
	Nasal septal perforation.	Very rare

As with other nasal sprays, unpleasant taste and smell and headache have been reported.

As with other nasal sprays, dryness and irritation of the nose and throat, and epistaxis have been reported. Nasal septal perforation has also been reported following the use of intranasal corticosteroids.

Systemic effects of some nasal corticosteroids may occur, particularly when prescribed at high doses for prolonged periods.

SYMPTOMS AND TREATMENT FOR OVERDOSE AND ANTIDOTE(S): There are no data available on the effects of acute or chronic overdosage with Flutinide Nasal Spray. Intranasal administration of 2 mg fluticasone propionate twice daily for seven days to healthy human volunteers has no effect on hypothalamo-pituitary-adrenal (HPA) axis function.

Inhalation or oral administration of high doses of corticosteroids over a long period may lead to suppression of HPA axis function.

PACKING: 15ml plastic spray bottle (120 doses).

STORAGE: Store below 30°C. Keep out of reach of children. Jauhi daripada kanak-kanak. Keep bottle in upright position.

Recommended shelf-life: 3 years.

Manufactured by:

HOE Pharmaceuticals Sdn. Bhd.

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